

EVIDENCE LOG

For PTSD (Post-Traumatic Stress Disorder) VA Disability Claims

THE SMOKING GUN EVIDENCE

After years of navigating the VA claims process—as veterans ourselves—we discovered something that changes everything: **the single most powerful piece of evidence you can submit isn't officially part of a Fully Developed Claim.**

"No more convincing. Just corroboration."

When you walk into a C&P exam without this documentation, the examiner's job is to **investigate whether your condition exists and how severe it is**. They're playing detective. You're trying to convince them.

When you walk in WITH a properly completed Evidence Log, the dynamic completely shifts. The examiner's job becomes **corroborating what you've already documented**. They're not investigating—they're verifying. That's a fundamentally different exam.

HOW VA RATERS ACTUALLY EVALUATE YOUR CLAIM

Every VA disability rating—regardless of condition—is based on three core factors. Raters are trained to look for evidence of these specific elements:

FACTOR	WHAT IT MEANS	WHAT RATERS LOOK FOR
FREQUENCY	How often do your symptoms occur?	Daily? Weekly? Constant? With specific triggers?
SEVERITY	How intense are your symptoms when they occur?	Flashback intensity? Duration? Dissociative episodes?

FUNCTIONAL IMPACT	How do symptoms affect your daily life?	Can't work? Can't sleep? Avoid public places? Relationship breakdown?
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Why This Evidence Log Works: The template you're about to use is specifically designed to capture all three factors in a format that's easy for raters to process. When a rater opens your file and sees 30 days of documented symptoms with frequency, severity, and functional impact clearly recorded, they don't have to guess or interpret. The evidence speaks for itself.

DO NOT FILE ANY CLAIM UNTIL THIS LOG IS COMPLETED AND INCLUDED WITH YOUR SUBMISSION

PTSD: CONDITION-SPECIFIC GUIDANCE

PTSD claims are evaluated under 38 CFR § 4.130, Diagnostic Code 9411. The VA uses the **General Rating Formula for Mental Disorders**, focusing on social and occupational impairment.

Key Stressor Categories (38 CFR § 3.304(f)):

STRESSOR TYPE	EVIDENCE NEEDED	EXAMPLES
Combat Engagement	Lay statement alone is sufficient	Direct firefights, ambushes, patrols under fire
Fear of Hostile Activity	Lay statement + VA psych confirmation	IEDs, incoming mortar/rocket fire, convoy threats
Military Sexual Trauma (MST)	Behavioral change markers in records	Assault, harassment — look for transfer requests, performance drops
Non-Combat Stressor	Full corroboration required	Accidents, witnessing death in non-combat settings

Critical: For combat veterans and those who experienced fear of hostile military/terrorist activity, **your lay testimony alone can establish the stressor** if it is consistent with the places, types, and circumstances of your service. You do NOT need official records of the specific incident.

What to Log Every Day:

- **Intrusion Symptoms:** Flashbacks, nightmares, intrusive memories — note triggers, duration, and intensity (1-10).
- **Avoidance:** Places, people, conversations, or activities you avoided and WHY.
- **Hyperarousal:** Hypervigilance, exaggerated startle response, sleep disturbance, irritability/anger outbursts.
- **Negative Cognition/Mood:** Feelings of detachment, guilt, shame, emotional numbness, inability to feel positive emotions.
- **Dissociative Episodes:** Lost time, feeling detached from reality, flashbacks where you "leave" the present.

Rating Criteria:

RATING	IMPAIRMENT
30%	Occasional decrease in work efficiency due to depressed mood, anxiety, suspiciousness, chronic sleep impairment.
50%	Reduced reliability and productivity. Flattened affect. Panic attacks more than once a week. Difficulty understanding complex commands. Impaired judgment.
70%	Deficiencies in most areas (work, family, thinking, mood). Suicidal ideation. Obsessional rituals. Near-continuous panic/depression. Inability to establish relationships. Impaired impulse control.
100%	Total occupational and social impairment. Gross impairment of thought/communication. Persistent delusions or hallucinations. Persistent danger of hurting self or others. Inability to perform basic self-care. Disorientation. Memory loss for own name/occupation.

COMPLETED EXAMPLE: 7-DAY EVIDENCE LOG

Below is an example of a properly completed PTSD Evidence Log. Notice how each entry captures frequency, severity, AND functional impact.

DATE	TIME	SYMPTOMS & SEVERITY	FUNCTIONAL IMPACT
Mon 1/6	0230	Nightmare — relived convoy IED blast. Woke drenched in sweat, heart pounding. Severity 8/10. Could not fall back asleep.	Called in sick to work. Stayed home all day. Checked perimeter of house 3 times. Wife slept in guest room.

Tue 1/7	1400	Flashback triggered by car backfire in parking lot. Dropped to ground, dissociated for ~30 seconds. Hands shaking for 20 min after. Severity 9/10.	Left grocery store without buying anything. Sat in car for 45 minutes. Drove home using back roads to avoid traffic. Canceled dinner plans.
Wed 1/8	All day	Emotional numbness. Could not feel anything — not happy, not sad. Flat. Kids asked to play, felt nothing. Severity 6/10.	Sat on couch all day. Did not shower. Did not eat until 1800. Avoided eye contact with family.
Thu 1/9	2100	Hypervigilance at daughter's school event. Sat with back to wall, scanned exits repeatedly. Heart rate elevated entire time. Severity 7/10.	Left event 30 min early. Argument with wife in car — she said I "ruined it." Slept in truck in garage.
Fri 1/10	0300	Night terror — woke up swinging. Nearly struck wife. She was screaming. Severity 9/10.	Wife moved to guest room permanently. Called VA crisis line at 0400. Could not work — exhausted and ashamed.
Sat 1/11	1100	Road rage incident — driver cut me off, I followed them for 2 miles before realizing what I was doing. Impaired impulse control. Severity 8/10.	Pulled over shaking. Called buddy from unit to talk me down. Did not drive again rest of day. Avoided family.
Sun 1/12	All day	Complete withdrawal. Did not leave bedroom. Intrusive thoughts about deployment. Guilt about things I saw/did. Severity 7/10.	Missed church. Did not eat. Did not speak to family. Wife left food outside door.

YOUR EVIDENCE LOG: 30-DAY TEMPLATE

Instructions: Complete one row per day for at least 30 consecutive days. Be specific — include times, triggers, severity (1-10), duration, and exactly how each symptom affected your ability to function. Describe your WORST days honestly.

VETERAN NAME: _____ **CLAIM #:** _____
CONDITION: PTSD (Post-Traumatic Stress Disorder) (DC 9411)
START DATE: _____

DATE	TIME	SYMPTOMS & SEVERITY	FUNCTIONAL IMPACT
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Day 1			
Day 2			
Day 3			
Day 4			
Day 5			
Day 6			
Day 7			
Day 8			
Day 9			
Day 10			
Day 11			
Day 12			
Day 13			
Day 14			
Day 15			
Day 16			
Day 17			
Day 18			
Day 19			
Day 20			
Day 21			
Day 22			

Day 23			
Day 24			
Day 25			
Day 26			
Day 27			
Day 28			
Day 29			
Day 30			

Note: Print additional copies if needed. The more detailed and consistent your log, the stronger your claim.

TIPS FOR MAXIMUM IMPACT

1. **Log EVERY day** — even "good" days. Write "Manageable today — mild hypervigilance at store, 3/10." Gaps look like you're exaggerating bad days.
2. **Use specific times** — "0300" is more credible than "nighttime."
3. **Name the trigger** — "Car backfire" is stronger than "loud noise."
4. **Quantify severity** — Use 1-10 scale consistently.
5. **Show functional impact** — "Missed work," "wife slept elsewhere," "kids asked what's wrong" — these are what drive ratings.
6. **Don't minimize** — Your worst days ARE your reality. Document them.
7. **Bring this log to your C&P exam** — Hand it directly to the examiner.